

Clinical Session Report

GhaithAI · AI-Assisted Clinical Decision Support

DRAFT · PENDING APPROVAL

Session Date: 25 Jun 2026

Report ID: 11111111

[GhaithAI CLINICAL SUPPORT SYSTEM - NOT A DIRECT MEDICAL DIAGNOSIS. THIS AUTO-GENERATED REPORT IS FOR LICENSED CLINICIAN REVIEW ONLY. DO NOT DISCLOSE TO PATIENT WITHOUT CLINICAL INTERPRETATION.]

[Patient data processed under explicit consent for AI-assisted clinical support, recorded in system logs at the start of the session.]

PATIENT DEMOGRAPHICS & BASELINE

Name: Omar Hassan

Gender: Male

Age: 28

Session Type: Online

Stress Level: High

Sleep Quality: Poor (3-4 hours/night)

Prior Therapy: Yes

Current Meds: No

Primary Intake Concern: Worsening depressive spectrum features, feeling of regression over past 2 weeks.

ATTENDING CLINICIAN

Clinician: Dr. Ahmed Yassin | Specialty: Consultant Psychiatrist

RISK ASSESSMENT

Overall Risk Tier: **MEDIUM**

Suicidal Ideation: **IDENTIFIED**

DETAILS

IDENTIFIED — Patient reports passive SI. [VERBATIM: "يا ريت عربية نقل تخبطني وأخلص"] [VERBATIM: "نفسي أنا وما أصحاش"] [VERBATIM: "كبير... لو أنا مش موجود هيرتاحوا Load بحس إني"] No active plan, intent, or means identified in transcript. Denies active self-harm thoughts. [STATED]

NARRATIVE & PROTECTIVE FACTORS

Denies any self-harm actions or urges. [VERBATIM: "عمري ما فكرت أمسك حاجة وأأذي نفسي"] [STATED] [NO HARM-TO-OTHERS IDEATION IDENTIFIED IN TRANSCRIPT] Religious faith ("أنا راجل بصلي وعارف ربنا"), fiancée support, insight, willingness to engage in treatment, future orientation (concern about work performance). Protective factors are identified in this session. [STATED] Passive suicidal ideation with burden ideation, marked functional impairment, recurrent episode, but no active plan/intent identified, strong protective factors, and good insight.

REQUIRED CLINICIAN ACTIONS

[ORANGE ESCALATION: PATIENT IS CLASSIFIED AS MEDIUM SUICIDE/SELF-HARM RISK. CLINICIAN ACTIONS REQUIRED:

1. Complete collaborative safety plan and review emergency contacts.
2. Schedule close follow-up appointment within 72 hours.
3. Provide patient with crisis contact numbers (16394 / 080-8888332).]

CLINICAL SUMMARY (SOAP)

S — Subjective

Patient reports 2-week worsening of depressive spectrum features, with low energy, poor concentration, insomnia, anorexia, weight loss, and passive death wishes, in context of work stress.

O — Objective

MSE notable for features consistent with depressed mood, appears constricted affect, passive SI, good insight. RISK_TIER: MEDIUM.

A — Assessment

Recurrent mood-related presentation with anxious distress and passive SI, triggered by work stress, with good insight and protective factors identified in this session.

P — Plan

1. DIAGNOSTIC: Administer PHQ-9, C-SSRS for suicide risk monitoring. 2. THERAPEUTIC: Initiate CBT targeting negative cognitions and burden ideation. 3. SAFETY: MEDIUM risk — schedule 24-hour follow-up call, develop safety plan including crisis contact, involve fiancée with consent. 4. FOLLOW-UP & REFERRAL: Weekly therapy

sessions, psychiatric follow-up in 1 week, encourage resumption of gym and sleep hygiene.

CHIEF COMPLAINT

Primary Concern

Worsening depressive spectrum features, feeling of regression over past 2 weeks.

Duration

2 weeks (current episode); prior episode 4 years ago.

Episode Type

Recurrent.

Secondary Complaints

[VERBATIM: "مش أحسن حاجة. الأسبوعين اللي فاتوا كانوا نقال أوي عليا. حاسس إني يرجع لورا بدل ما أتقدم."] Work performance decline, brain fog, sleep disturbance, appetite loss, weight loss, anhedonia, passive death wishes.

HISTORY OF PRESENTING ILLNESS (HPI)

Narrative

Onset 2 weeks ago, gradual worsening in context of high work stress (software release). Prior episode 4 years ago during COVID-19 job loss, treated with medication for 8 months, self-discontinued after finding new job. Current episode characterized by low energy, poor concentration, insomnia, anorexia, weight loss, anhedonia, and passive death wishes; stressor is fear of job loss reminiscent of past trauma.

Functional Impact

Work/Academic: Severe decline, unable to write code, performance noted by manager. Sleep: Initial insomnia, non-restorative sleep, 3-4 hours/night. Appetite: Decreased, weight loss 5 kg in 1 month. Energy/Psychomotor: Low energy, stopped gym for 3 weeks. Interpersonal: Feels burden to fiancée and family. Self-care: Likely reduced due to low energy. Cognition: Brain fog, zero concentration.

Past Psychiatric & Medical History

Prior episode consistent with depressive spectrum features 4 years ago, treated with medication (SSRI and benzodiazepine) for 8 months, self-discontinued. No other medical history mentioned. [STATED]

Current Medications & Substances

Prior treatment: SSRI 10mg and benzodiazepine for sleep, 8 months, self-discontinued. Currently no medications. [STATED]

MENTAL STATE EXAMINATION (MSE)

Appearance & Behavior

NOT_ASSESSABLE

Speech, Mood & Affect

Coherent, with sighing. [INFERRED: sigh indicates distress] Features consistent with depressed mood. [VERBATIM: "أنا تعبت" "يا دكتور"] [STATED] Appears constricted. [INFERRED: from sigh and depressive content]

Thought Process & Content

Coherent, goal-directed. [INFERRED: from logical responses] Preoccupation with work failure, worthlessness, burden ideation, passive death wishes. No psychotic content. [STATED] [NOT MENTIONED: psychotic content]

Perception & Cognition

NOT_MENTIONED Subjective brain fog, poor concentration. [STATED] Formal testing not done. [NOT ASSESSABLE FROM TRANSCRIPT]

Insight & Judgement

Good; recognizes symptoms, seeks help, agrees to treatment. [STATED] Appears intact; weighs treatment options, expresses concern about medication side effects. [STATED]

CLINICAL FORMULATION

Narrative

Predisposing: History of prior episode consistent with depressive spectrum features, possible genetic vulnerability, high-achiever personality. [INFERRED: from recurrent episode and work stress sensitivity] Precipitating: Work stress, fear of job loss reminiscent of past trauma. [STATED] Perpetuating: Sleep deprivation, poor nutrition, social withdrawal, negative cognitions (burden, worthlessness). [INFERRED: from symptoms] Protective: Religious faith, supportive fiancée, insight, prior positive response to treatment. [STATED] A 20-something male with recurrent episodes consistent with depressive spectrum features triggered by work-related stress and fear of job loss, mirroring past trauma. Current episode marked by neurovegetative symptoms, cognitive impairment, and passive suicidal ideation. Protective factors include strong social support and religious coping, but perpetuating factors like sleep disruption and negative self-appraisal maintain the episode.

Differential Considerations

- 1. Mood-related presentation with anxious distress
- 2. trauma-related features (fear of job loss reminiscent of past unemployment)
- 3. rule out adjustment-related distress features with mixed anxiety and depressed mood
- 4. consider recurrent depressive spectrum features

PHARMACOLOGICAL NOTE

Pharmacological considerations, if clinically indicated, are to be evaluated exclusively by the supervising licensed physician or psychiatrist. No medication recommendations are made or implied by this AI-generated report.